Feet For Life Centers Inc.
314 983 0303
8637 Delmar Blvd
Saint Louis MO 63124-1906

Patient: Graham Scanlon
Date: 10/28/2026

Type of Note:
Progress Note

Reason For Visit
Date of onset: 2020 and Date of onset: 3/2026.
2nd MTP joint pain.
Bone contusion on lower fibula/ the knub on the outside of the left ankle.
Conservative measures-pads, orthotics, offloading, steriod shot, oral steroids, PT, contrast therapy, toe speaders.
R.I.C.E.

Notes
Date of Service: 10/28/2026
Patient: Graham Scanlon
Provider: Michael Horwitz, DPM
Visit Type: new; in-person
Chief Complaint: Left foot pain near the second toe joint
Subjective
1: Left foot pain
Seven months of pain near the second toe joint, initially a "pebble feeling" under the joint. Pain varies from 4 to 10 out of 10. Sharp pain occurs intermittently, including with firm pressure. Recently developed nighttime throbbing that can become "burning"; occasionally feels the toe "vibrate." No shooting pain, dull aching, or numbness between the toes. Prior X-rays reportedly showed no fracture; arthrogram reportedly showed no plantar plate tear. Previously diagnosed with capsulitis; patient reports MRI showed capsulitis. One cortisone injection at Horizon Foot and Ankle did not reduce peak pain but helped it settle faster. Walking tolerance increased from 15-20 minutes to about 30 minutes after injection.
Medications:
Allergies:
Objective
Vitals:
Exam:
- Left foot: Tenderness near the second toe joint. Minimal pain with rolling the foot. Limited first-ray hypermobility; preserved arch.
Data Reviewed:
- Left foot X-rays today, AP, lateral, and oblique: Long first ray with mild arthritic changes at the great toe joint. Lesser metatarsal parabola within normal limits; no cortical disruption. Normal first-ray declination on lateral view. Joint spaces preserved, with slight flattening of the metatarsal head.
Assessment & Plan
1. Left second toe joint capsulitis: persistent
Assessment: Symptoms primarily suggest capsulitis, with possible adjacent nerve irritation or altered nerve course. Symptoms are not typical of a diagnosable Morton's neuroma. Limited response to one cortisone injection; prior arthrogram reportedly showed no plantar plate tear.
- Diagnostics: No additional MRI recommended at present.
- Counseling: Discussed possible nerve involvement from joint inflammation or compensation. Discussed uncertainty of cryotherapy benefit without a clear neuroma diagnosis. Reviewed risks of repeat cortisone injections, including capsular stretching and hammertoe. Discussed metatarsal shortening with a Weil osteotomy as a potential option for persistent pain. Discussed limited confidence in platelet-rich plasma or biologics for this inflammation.
- Non-pharmacologic: Discussed cold laser treatment. If the newer MIS laser is available in January and pain persists, consider twice-weekly treatment for 10 sessions, with response assessed after four sessions. The currently available M7 laser is an alternative, though effectiveness for foot conditions is uncertain. Send laser information to the patient.
- Follow-up: Reassess if pain persists; Return precautions: Seek earlier evaluation for worsening pain, new numbness or weakness, or difficulty bearing weight.
mh

History of Present Illness
Graham Scanlon is a 27 year old male.
- Allergy list reviewed - Problem list reviewed - Medication list reviewed

Current Medication
Medication list reviewed.

Past Medical/Surgical History
Diagnoses:
Anemia

Social History
Tobacco use and exposure: No tobacco use. Never smoked. Smoking status: Never smoker.
Alcohol: Never drank alcohol.

Physical Findings
- Vitals taken 10/28/2026 12:43 pm
BP-Sitting 142/75 mmHg
Height 73 in

Weight 145 lbs
Body Mass Index 19.1 kg/m2
Body Surface Area 1.9 m2

Discussed
- Education and instructions;
- Instructions for patient;
- Education and counseling

Plan
StartCited - Enthesopathy, unspecified
ML7: ML7
EndCited
Practice Management
Clinical summary provided to patient.

- **Clinical Note**, Last documented on 10/29/2026; 11:04 AM; Feet For Life Centers Inc.; Michael Horwitz